



P45 · 40 PAGES, PRINTABLE + FILLABLE

Twin Postpartum, the First Six Weeks

A forty-page postpartum planner for twin parents in the first six weeks home. Day-by-day through week one, then week-by-week through week six. Includes the two-baby feed-and-diaper log designed for side-by-side tracking, the "who has which baby" rotation page, the pump-with-twins guidance by purpose only, visitor management specifically for twin parents (different than singleton), a partner page, the "if still NICU" page, and an honest "I am not okay" page because postpartum mental-health rates are higher in twin parents.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

A note before you start

You came home with two of them.

The first week is mostly survival. The second is when you start to lose the thread of what day it is. The third is when the visitors start to thin out. The fourth is the one where you wonder if you will ever feel rested again. The sixth is the appointment that closes this chapter.

This planner is for that stretch. Not for what twin parenting will look like at six months, when you have your rhythm. For now. For the six weeks where the rhythm is being built, on no sleep, with two of them needing you at the same time.

It is also for the partner. The first six weeks of twin postpartum is a two-person job for nearly everyone. The pages that have a "partner" header are for the second adult to read and write in.

Soothemade

This is a planning tool, not medical advice. For specific concerns about recovery, feeding, mental health, or your babies' health, please follow your obstetric and pediatric providers' guidance. The planner does not name medications. The mental-health crisis line page is at the back. bookmark it now.

What is different for twin postpartum

The list, in plain language. None of these is a “but you can do it” framing. They are the realities.

Feeding takes twice as long, or differently. Whether you are exclusively breastfeeding, exclusively bottle-feeding, or mixing, the feeding schedule for two is its own logistical structure. Tandem feeds, alternating feeds, one parent on each baby, none of those work the same as one-baby feeding.

Sleep is more disrupted, statistically. Two babies wake on their own schedules. Some twin parents work hard to synchronize them. Some do not. Both are valid. The math is “fewer hours of unbroken sleep” either way.

Recovery for the parent is sometimes longer. Multifetal pregnancies often deliver earlier, sometimes with more complications. A twin postpartum recovery is its own thing. Your provider knows the specifics.

Mental health is more affected. Postpartum depression and postpartum anxiety occur at higher rates in twin parents than singleton parents. This is well-documented. It is not a moral failing. It is a known pattern. The planner has explicit pages for early-warning signs.

The village is heavier. Twin parents need more help than singleton parents. The visitor balance is different. You need more hands and you need them more often.

Visitor expectations are sometimes weird. People treat twins as a curiosity. “Oh my god, two.” You will hear this many times. The visitor management page has scripts.

Costs are higher. Two of many things. The financial weight surfaces faster.

Comparison is harder. Two babies the same age, in the same home. They will be different in everything. The comparison instinct is strong. The planner has a permission slip for that.

Where I am right now, on day ____:

What feels harder than I expected:

What feels easier:

The first 24 hours home

For most twin parents, the first day home is the day everyone is still adjusting. The car ride home. The hand-off from the car seat. The first feed at home.

ARRIVAL HOME

Date + time: _____

Who carried which baby in: _____

THE FIRST FEED AT HOME

Baby A: time _____ length _____ side / bottle _____

Baby B: time _____ length _____ side / bottle _____

Notes:

THE FIRST DIAPER AT HOME (one each)

Baby A: time _____ wet / dirty _____

Baby B: time _____ wet / dirty _____

THE FIRST NAP

Baby A: started _____ ended _____

Baby B: started _____ ended _____

Where each slept:

THE PARENT

Have I eaten? ☐ Yes ☐ No (eat)

Have I drunk water? ☐ Yes ☐ No (drink)

Pain level (1-10): _____

Anything I need to call the provider about tonight?

THE PARTNER

Are they overwhelmed? ☐ Yes ☐ No

Have they eaten? ☐ Yes ☐ No

What do they need before bed?

THE NIGHT PLAN

Who is on which baby: _____

What time the rotation switches: _____

Day 2

DATE: _____

FEEDS (rough, no need for perfection)

Baby A: ____ feeds, total ____ minutes / oz

Baby B: ____ feeds, total ____ minutes / oz

DIAPERS

Baby A: ____ wet ____ dirty

Baby B: ____ wet ____ dirty

SLEEP

The longest stretch I slept: _____ hours

The longest stretch Baby A slept: _____ minutes

The longest stretch Baby B slept: _____ minutes

PARENT RECOVERY

Pain (1-10): _____

Bleeding (heavy / moderate / light): _____

Eaten: ☐ breakfast ☐ lunch ☐ dinner

Showered: ☐ yes ☐ no

Stairs: ☐ avoided ☐ took them

WHAT IS WORKING TODAY:

WHAT IS NOT:

Day 3

DATE: _____

FEEDS

Baby A: ____ feeds Baby B: ____ feeds

DIAPERS

Baby A: ____ wet ____ dirty Baby B: ____ wet ____ dirty

SLEEP

My longest stretch: _____ hours

Did I nap during the day: ☐ yes ☐ no

PARENT

Pain (1-10): ____ Bleeding: _____

THE THING THAT SURPRISED ME ABOUT TODAY:

Day 4

DATE: _____

FEEDS

Baby A: ____ feeds Baby B: ____ feeds

DIAPERS

Baby A: ____ wet ____ dirty Baby B: ____ wet ____ dirty

SLEEP

My longest stretch: _____ hours

PARENT

Pain (1-10): ____ Mood (1-10): ____

WHAT I AM ASKING SOMEONE TO DO FOR ME TOMORROW:

Day 5

DATE: _____

FEEDS

Baby A: ____ feeds Baby B: ____ feeds

DIAPERS

Baby A: ____ wet ____ dirty Baby B: ____ wet ____ dirty

SLEEP

My longest stretch: _____ hours

PARENT

Pain: ____ Mood: ____ Anything new I'm noticing:

A NOTE: day 4-5 is often when the "baby blues" hit. Tears for no reason. Big emotions. This is common and usually passes within a week. If it does not, the mental-health page at the back is the next step.

Day 6

DATE: _____

FEEDS

Baby A: ____ feeds Baby B: ____ feeds

DIAPERS

Baby A: ____ wet ____ dirty Baby B: ____ wet ____ dirty

PARENT

Pain: ____ Mood: ____ Energy: ____

ONE THING THAT HELPED ME TODAY:

Day 7, the first week complete

DATE: _____

ONE WEEK HOME. WHAT I HAVE LEARNED ABOUT BABY A:

ONE WEEK HOME. WHAT I HAVE LEARNED ABOUT BABY B:

ONE WEEK HOME. WHAT I HAVE LEARNED ABOUT MYSELF:

THE HARDEST DAY OF THIS WEEK WAS: _____

Because:

WHAT I WANT THE SECOND WEEK TO HAVE MORE OF:

WHAT I WANT IT TO HAVE LESS OF:

Week 2

DATES: _____ to _____

FEEDS, ROUGH AVERAGE PER DAY

Baby A: ____ feeds Baby B: ____ feeds

NIGHT

My average longest stretch of sleep: _____ hours

Who is doing the night shift: _____

The current night-shift rotation:

PARENT BODY

Pain (1-10): _____

Bleeding: _____

Anything I am calling the OB about: _____

PARENT MIND

Mood across the week (1-10): _____

What I notice in myself this week:

THE VILLAGE

Who is coming this week: _____

What they are doing: _____

Who I need to ask for next week: _____

ANYTHING I AM HOLDING THIS WEEK THAT I HAVE NOT TOLD ANYONE:

Week 3

DATES: _____ to _____

FEEDS

Pattern is starting to look like:

NIGHT

Best night this week: _____

Worst night this week: _____

PARENT BODY

Pain: ____ Anything still concerning:

PARENT MIND

How the partner is doing:

How I am doing:

THE VILLAGE

This week was: ☐ enough help ☐ not enough ☐ too much

ONE WIN FROM THIS WEEK:

ONE THING I AM STILL FIGURING OUT:

Week 4

DATES: _____ to _____

FEEDS

What is the current rhythm:

NIGHT

My average longest stretch of sleep: _____ hours

PARENT BODY (week 4 is often the postpartum mood-shift week)

How the body feels overall:

PARENT MIND

How is the baby blues / "real" sadness / anxiety?

- ☐ Improving
- ☐ Same
- ☐ Worse (call the provider if worse)

The four-question PHQ-style check (not a diagnosis, just a noticing):

- Most days, am I able to find any pleasure in things I used to enjoy?
Yes / No / Some days
- Most days, do I feel sad or hopeless?
Yes / No / Some days
- Am I sleeping when I can sleep, or am I lying awake?
Sleeping / Awake / Mixed
- Am I able to eat? Yes / No / Some

If two or more of these are pointing in the "not okay" direction,
the mental-health page at the back has the lines to call.

THE VILLAGE

Who is still coming: _____

Who is fading: _____

Who do I need now that I did not need in week 1:

Week 5

DATES: _____ to _____

FEEDS

The pattern is roughly:

NIGHT

Are they starting to sleep longer stretches?

- ☐ Yes (good)
- ☐ Not yet (also normal)
- ☐ Worse, actually (mention to pediatrician)

THE PARENT

Pain (1-10): _____

Mood (1-10): _____

Body recovery (1-10): _____

ANYTHING I WANT TO REMEMBER FROM THIS WEEK:

Week 6, the closing week

The six-week postpartum appointment is often around now. This is the closing chapter of the planner.

DATES: _____ to _____

THE SIX-WEEK APPOINTMENT

Date: _____

Provider: _____

Questions I want to bring:

1. _____

2. _____

3. _____

4. _____

Notes from the appointment:

LOOKING BACK ON SIX WEEKS

What I would tell the version of me on day one:

What I am most proud of:

What I am still figuring out:

What I have stopped being scared of:

What I am still scared of:

Print this page as often as you need to.

Code for feed column: BR = breast (which side), BO = bottle (how many oz), MIX = mixed
Code for diaper columns: W = wet, D = dirty (mark each column with /1 for a tick)

The “who has which baby” rotation page

Twin parents need explicit rotation. The “you got this one, I’ll take the other” arrangement that happens in the moment also needs a planned version.

THE OVERNIGHT ROTATION

First shift: ____ PM to ____ AM

Adult on duty: _____

Which baby they have (if separating): _____

Second shift: ____ AM to ____ AM

Adult on duty: _____

Which baby they have: _____

Third shift (if there is one):

Adult on duty: _____

ARE WE SEPARATING THE BABIES OR ROTATING ONE PARENT ON BOTH?

- ☐ Separating: one adult per baby, switch nightly
- ☐ Together: one adult on both, the other adult sleeps
- ☐ Tandem-feed: both babies on one adult during shift
- ☐ Something else: _____

WHEN THE ROTATION FAILS

What do we do when an adult is too sick / too exhausted / too out of it to take their shift?

Who is the backup adult? _____

At what point do we call them?

Pumping with twins

This is for the parent who is pumping for twins, exclusively or in combination with direct breastfeeding. The clinic, the IBCLC, and your provider are the people who tell you which medications, supplements, or supply-support measures apply to you. This page is logistics only.

PUMP TIMES PER DAY (the clinic told you a target)

Number of times per day target: _____

How long each pump session: _____

Where I will pump: _____

THE PUMP SETUP

- ☐ A dedicated spot with the pump, parts, bottles, breastpads, water
- ☐ A timer or app
- ☐ A way to multitask (book, phone, headphones)
- ☐ A photo of the babies (sometimes helps with letdown)

THE LOGISTICS

How I store the milk:

How I label it:

Who washes the pump parts:

When I order more parts / replace gear:

IF SUPPLY IS A QUESTION

This is a clinic conversation. The IBCLC, the lactation consultant at the hospital, the pediatrician for the babies, your OB for you, all of them are the right people to call. Not the internet.

Number to call: _____

Visitor management for twin parents

Twin visitors are different from singleton visitors. People come to see the curiosity. Some of them are helpful. Some of them are not.

WHO TO LET IN, AND WHY

THE HELPER VISITOR (yes):

- Brings food
- Does dishes
- Holds a baby while you eat
- Stays in the kitchen, not the nursery
- Leaves when you give the signal

THE PERFORMANCE VISITOR (limit):

- Wants photos of the twins
- Wants to hold both at once
- Stays for two hours
- Does not help

WHAT TO SAY TO THE PERFORMANCE VISITOR

"We are doing 20-minute visits this month. Thank you for understanding."

"We are not posting photos yet. Thank you for not posting any you took."

"We need help with [specific task] more than holding right now. Are you open to [task]?"

"We are taking a quiet day. Can we reschedule for [later date]?"

THE WEEKLY VISITOR CAP (worth setting)

This week's visitor count: _____

What I will say to the eighth person who asks:

Whose visits ENERGIZE me:

Whose visits DRAIN me:

(Reduce the latter.)

Partner page

For the second adult. The first six weeks of twin postpartum needs both of you, in a way that singleton postpartum sometimes does not.

THE WORK SHARE, HONEST

What the recovering parent is doing right now that you may not see:

- Healing from delivery (often a longer recovery than singleton)
- Producing milk if breastfeeding (calorically demanding)
- Carrying the hormonal shift (which is hard, even when nothing is "wrong")
- Sleeping less than the math suggests because they are still alert to the babies even during their "off" shift

What you (the partner) are doing that you may not be acknowledging:

- Picking up nearly everything outside of feeding
- Managing the visit calendar
- Doing two diapers for every one feed
- Carrying your own grief and shock (this is real, even though you did not deliver)

THIS IS A SHARED COLLAPSE, NOT A ONE-SIDED ONE

WHAT TO WATCH FOR IN YOUR PARTNER

If any of these is true, the call to the provider's office is yours to make, on your partner's behalf if needed:

- ☐ They are crying more than once a day, for more than 20 minutes at a stretch, past week 2
- ☐ They have not eaten a full meal in 48 hours
- ☐ They are not sleeping when the babies sleep, for more than a few nights
- ☐ They have said something like "the babies would be better off without me" (this is a 988 / 1-833-852-6262 call)
- ☐ They are not interested in the babies in a way that feels new
- ☐ Their anxiety is consuming hours of every day
- ☐ Your gut is telling you something

WHAT YOU CAN SAY

- "I will take this one. You sit."
- "You are doing the hardest thing I have ever seen."

- "I am calling the provider on your behalf."
- "We are not going to figure this out tonight. Let me hold both of them for an hour while you sleep."

WHAT TO NOT SAY

- "Just relax."
- "It will get better." (true, but not a fix-it sentence)
- "Other people do this with no help." (this is unhelpful and usually untrue)
- "Why are you crying?"

YOUR OWN MENTAL HEALTH

Partner postpartum depression and anxiety is real. The rates are not as high as for the recovering parent, but they are real.

If you are noticing your own withdrawal, rage, dread, or sense of "I cannot do this," that is the moment to talk to your own provider too.

If still NICU

A meaningful share of twin parents are coming home with one baby or no babies, while the other(s) finish NICU time. This is a planner page for that.

WHO IS WHERE

Babies in NICU: _____

Babies home: _____

Estimated NICU discharge date: _____

(This can change daily. do not commit to it emotionally)

THE SPLIT-FAMILY LOGISTICS

Who goes to the NICU each day: _____

At what times: _____

How long: _____

Who stays home with the baby (if any): _____

Are we trading off? ☐ Yes ☐ No

The hardest part of this split:

THE EMOTIONAL PART

Twin NICU is a particular kind of hard. You may be home with one baby, holding them, feeling guilty that the other is in the hospital. You may be at the hospital, holding the NICU baby, feeling guilty that the home one is not getting you. Both are common. Neither is your fault.

What I am holding right now that I have not said out loud:

Who I can say it to:

P28 NICU PARENT PLANNER is the deeper companion if you have NICU time extending beyond the first week.

The “I am not okay” page

Twin postpartum has higher rates of postpartum mood disorders. This page is named honestly because the data is not subtle. PPD, PPA, postpartum OCD, postpartum rage, all of them present in twin parents at meaningfully higher rates than singleton parents.

QUESTIONS TO ASK MYSELF, HONESTLY, AT ANY POINT IN THESE SIX WEEKS

- Am I crying daily and unable to stop? ☐ Yes ☐ No
- Do I have intrusive thoughts about harm to me or to the babies? ☐ Yes ☐ No
- Have I had thoughts of "they would be better off without me"? ☐ Yes ☐ No
- Am I unable to sleep even when the babies are sleeping? ☐ Yes ☐ No
- Am I unable to eat? ☐ Yes ☐ No
- Am I disconnected from the babies in a way that feels not like me? ☐ Yes ☐ No
- Am I rage-filled in a way that scares me? ☐ Yes ☐ No
- Is my anxiety running my whole day? ☐ Yes ☐ No

IF YOU CHECKED ANY OF THE FIRST THREE, THE CALL IS NOW.

CRISIS / SUPPORT LINES

US, 988 Suicide and Crisis Lifeline (call or text)

US, Postpartum Support International HelpLine
1-833-852-6262 (call or text)

US, Maternal Mental Health Hotline (24/7)
1-833-943-5746

UK, Samaritans: 116 123

Canada, 9-8-8 Suicide Crisis Helpline

Australia, PANDA: 1300 726 306

YOUR PROVIDER

OB office: _____

After-hours OB: _____

The therapist I want to call if needed:

A NOTE: ASKING FOR HELP IS NOT WEAKNESS. THE RATES OF POSTPARTUM DEPRESSION IN TWIN PARENTS ARE HIGHER FOR REASONS THAT HAVE NOTHING TO DO WITH YOU PERSONALLY. THE TREATMENT IS GOOD. THE SOONER YOU START, THE FASTER YOU GET YOURSELF BACK.

A final letter

Six weeks.

You did it. Or you are still in it, reading this on week 4, and you are doing it. The planner ends at the six-week mark because that is a milestone, not because the work is over. The work is just shifting.

By week six, the babies are individuals to you. You can tell their cries apart. You know which one likes to be held a certain way. You know that there is one who screams during the bath and one who loves it. You know things about them that nobody else in the world knows.

The next phase is different. It is sleep training conversations (or not). It is starting solids. It is the dawn of social smiles, the first weight check that they have passed birth-weight by miles, the photos that get easier to take. The planner closes here because the next chapter is its own.

You did the hardest first six weeks of the first year. With two of them. With less sleep than you knew was possible. You showed up. They are here. You are here.

That is the planner. That is the work. That is enough.

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Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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